

MENTAL HEALTH

NCLEX STUDY GUIDE

Assessment • Priorities • Medications • Safety

1. Schizophrenia



Therapeutic communication: calm presence, active listening and safety assessment

Core assessment findings

- Hallucinations: auditory hallucinations are most common. Command hallucinations are the priority.
- Illusions: a real external stimulus is present but is misinterpreted or distorted.
- Delusions: fixed false beliefs not based in reality.
- Disorganized speech: neologisms, clang associations, word salad and tangential speech.
- Regression and concrete thinking may occur; abstract thinking can be impaired.

NCLEX PRIORITY Ask directly what the voices are saying. Acknowledge the client's experience without confirming the hallucination: "That must be difficult for you. I do not hear the voices."

Delusion type	Meaning / example
Grandiose	Exaggerated power or identity; e.g., claiming to be president.
Persecutory	Belief that one is the victim of a plot.
Ideas of reference	Belief that television, radio, phones or social media send personal messages.
Somatic	False belief involving the body or unusual physical powers.

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Disorganized	Catatonic	Paranoid
Silly or inappropriate laughing	Waxy flexibility; mutism	Suspiciousness and mistrust

Nursing management

- Reduce environmental stimulation and move the client to a calm area.
- Validate reality; do not reinforce hallucinations or delusions.
- Monitor early signs of escalating behavior.
- For paranoid clients, offer sealed or packaged foods when appropriate.
- Explain staff changes, vacations and absences in advance when possible.

Antipsychotic medications

Typical - more EPS	Atypical - key monitoring
Haloperidol; chlorpromazine; fluphenazine; perphenazine; thioridazine; trifluoperazine	Clozapine: decreased WBC/infection risk; olanzapine: increased glucose; quetiapine: increased cholesterol; risperidone: increased triglycerides; ziprasidone: QT prolongation; aripiprazole.

SAFETY Extrapyramidal effects are involuntary movements the client cannot control.

2. Mood Disorders



Mental-health care integrates mood, anxiety, sleep, treatment monitoring and recovery support

Major depression

- Anhedonia and suicidal ideation are major assessment findings.
- Ask directly about thoughts, intent and a plan to harm oneself.
- Warning signs include a sudden mood change, giving away possessions or pets, leaving a note or power of attorney, and no longer debating suicide.
- Spend time with the client and return when promised (fidelity).

Medication groups

Group	Examples / concern
Tricyclic antidepressants	Monitor adverse effects and overdose risk.
MAOIs	Selegiline, isocarboxazid, phenelzine, tranylcypromine.
SSRIs	Escitalopram, sertraline, citalopram, paroxetine, fluoxetine, fluvoxamine. Watch for serotonin syndrome.

Electroconvulsive therapy (ECT)

ECT induces a controlled seizure under medical supervision. The original notes identify fracture risk and caution with osteoporosis or alendronate therapy.

BOARD ALERT ECT is not ordinarily scheduled every day. Verify preparation, consent and post-procedure safety.

3. Anxiety, OCD and Trauma-Related Disorders

Anxiety and phobias

Anxiety disorders may persist for more than six months. Panic attacks are discrete periods of intense fear or discomfort that may be unexpected and incapacitating.

Term	Meaning
Mysophobia	Fear of dirt or germs
Monophobia	Fear of being alone
Ornithophobia	Fear of birds

Defense mechanisms

- Displacement: transferring feelings from the original source to a safer target.
- Projection: attributing one's own feelings or impulses to another person.
- Repression: involuntary blocking of a distressing event; suppression is voluntary.
- Sublimation: channeling an unacceptable impulse into a constructive activity.

BOARD PRIORITY Systematic desensitization progresses from least threatening to most threatening: discuss the animal, view a photo, watch a video, then approach the animal.

Obsessive-compulsive disorder

Obsessions are repetitive intrusive thoughts; compulsions are repetitive actions performed to reduce anxiety. Clomipramine may be prescribed.

Post-traumatic stress disorder

- May follow war or another traumatic experience and include flashbacks.
- Assess emotional detachment and suicidal or homicidal ideation.
- Listen and provide a safe, nonjudgmental presence.

4. Somatic and Dissociative Disorders

Disorder	Key point
Somatization	Multiple physical complaints; frequent provider changes (“doctor shopping”). Complex chronic cases may need case management.
Illness anxiety / hypochondriasis	Persistent fear of serious illness. Respond without judgment.
Conversion disorder	Unintentional, reversible neurologic symptoms linked to conflict; la belle indifférence may occur.
Factitious disorder	Intentional symptom production to assume the sick role. Munchausen by proxy is abuse and must be reported.
Malingering	Intentional symptom production motivated by external or personal gain.

Dissociative disorders

- Fugue: unexpected travel or leaving work/home with inability to recall identity.
- Dissociative amnesia: temporary inability to recall information related to trauma or disaster.
- Dissociative identity disorder: switching between identities; call the host by name and report which personality is present during handoff.
- Depersonalization: stay with the client during the episode.

5. Personality and Childhood Disorders

Cluster	Disorders and characteristics
A - odd/eccentric	Schizoid: isolation. Schizotypal: odd beliefs, thinking and speech. Paranoid: suspiciousness and mistrust.
B - dramatic/erratic	Set clear limits. Histrionic: seeks attention. Narcissistic: grandiosity and exploitation. Antisocial: history of conduct disorder and disregard for rules. Borderline: splitting (others are all good or all bad).

Conduct disorder

Serious, persistent violation of rules and others' rights; may begin around age 10-12.

Oppositional defiant disorder

Defiant, argumentative and oppositional behavior without the severe rights violations of conduct disorder.

6. Eating Disorders

Anorexia nervosa

Underweight, cyanosis, lanugo, numbness; electrolyte imbalance is the priority.

Bulimia nervosa

Binge eating followed by purging; calluses, parotid enlargement (“chipmunk cheeks”), financial food stress, hypokalemia and metabolic alkalosis.

Nursing interventions

- Encourage journaling.
- Supervise meals and for two hours afterward.
- Obtain daily weight at the same time, on the same scale, after voiding.
- Topiramate may be used for bulimia and is also an antiseizure medication used for migraine prevention.

7. Substance Use Disorders

CAGE SCREEN Cut down • Annoyed • Guilty • Eye-opener

Alcohol intoxication	Withdrawal (6-24 hr)	Chronic alcoholism	Relapse deterrence
CNS depression; thiamine (vitamin B1) IV, IV fluids, and dextrose if hypoglycemic.	CNS excitation; chlordiazepoxide or lorazepam, clonidine, and IV thiamine as ordered.	Wernicke-Korsakoff syndromes; treat/prevent with thiamine.	Disulfiram, acamprosate, or naltrexone may be prescribed.

Drug class	Intoxication / overdose	Withdrawal
CNS depressants / opioids	Miosis and respiratory depression. Activated charcoal may be ordered; naloxone reverses opioid overdose.	Nausea/vomiting; opioid withdrawal may include yawning, rhinorrhea and lacrimation.
CNS stimulants	Mydriasis and marked stimulation; severe toxicity may include ataxia.	Depression and fatigue.
Cannabis	Increased appetite / food cravings.	Monitor mood, sleep and coping.

8. Grief, Delirium and Dementia

Grief type	Key point
Normal	Usually less than 6 months; anniversary reactions may be normal.
Complicated	Persists beyond expected adaptation, often more than 6 months.
Anticipatory	Begins before an expected death or loss.
Disenfranchised	Loss is not openly acknowledged or socially supported, such as pet loss or miscarriage.
Acute	Immediate intense response; may occur with criminal death during prosecution.

Children's understanding of death

- Younger than 2: unable to understand death.
- Ages 3-5: may believe death is reversible or a punishment.
- Ages 6-9: begin to understand finality.
- After age 9: clearer death concept. Adolescents generally understand at an adult level.

Delirium and dementia

Delirium may result from an acute medical cause such as a urinary tract infection. Dementia assessment may include agnosia and sundowning syndrome.

Alzheimer stage	Typical findings
1 - mild	Forgetfulness and difficulty with lists/routines; may still be able to consent and learn.
2 - mild/moderate	Increasing confusion; teaching remains important.
3 - ambulatory dementia	Wandering, ADL loss, loss of family recognition, and failure to recognize self in a mirror. Safety is the priority in stages 3-4.

- Use reality orientation when appropriate.
- Encourage reminiscence.
- For wandering, use safe environmental controls such as complex door locks positioned high and follow facility policy.

FINAL NCLEX FOCUS Prioritize safety, suicide assessment, command hallucinations, electrolyte imbalance, respiratory depression and acute changes in cognition.